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The stigmatization of prolonged grief disorder and disenfranchised grief: A vignette-based experimental study

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ABSTRACT

People with prolonged grief disorder (PGD) are at risk of public stigma, but research has yet to examine whether stigma is shaped by different types of relationship losses. In an experimental study, we asked participants ($N=306$) to read three scenarios in which targets lost their romantic partner, child, or companion animal. Targets with PGD (vs. integrated grief) elicited more negative emotional reactions and attributions, and their experiences were perceived as less legitimate. Targets who lost their companion animal (vs. other relationship losses) were perceived as the most sensitive and their grief as the least legitimate, but they also elicited the lowest prosocial and fear reactions and were perceived as the warmest. Lastly, targets with PGD who lost their companion animal (vs. other relationship losses) elicited more negative emotional reactions and attributions, and their experiences were perceived as less legitimate. Implications and suggestions for future studies are discussed.

Introduction

The death of a loved person (e.g., a romantic partner or a close family member) is among the most stressful and challenging life events (Doering & Eisma, 2016; Matthews & Marwit, 2004). Although most people are likely to cope with the distress caused by their losses and reintegrate into their lives (He et al., 2014; Pan & Liu, 2021), grief symptoms can become more intense and persistent for some people. A recent cross-national analysis indicated that around 13% of the bereaved people in a sample of 20,347 participants from 16 countries (e.g., UK, Germany, France, USA) met the diagnostic criteria of prolonged grief disorder (PGD; Comtesse et al., 2024). Cases of PGD¹ can have deleterious psychological and physical consequences that include increased risk of cancer, cardiovascular diseases, sleep disturbances, and reduced quality of life (Simon, 2013; Szuhany et al., 2021). Research has shown that certain individual and social determinants can increase the risk of more severe grief experiences, including having insecure attachment styles (Currier et al., 2015), higher anxiety (Piper et al., 2001), substance use problems (Parisi et al., 2019), and weaker social support systems (Levi-Belz & Lev-Ari, 2019). In addition to individual and social factors, certain

contextual determinants can also increase the risk of more severe grief experiences, including losing someone after a violent death (e.g., homicide, suicide, fatal accident; Nakajima et al., 2012) and having a more emotionally intimate relationship with the deceased (Lobb et al., 2010).

Research has already shown that stigmatizing beliefs and behaviors toward mental illnesses (e.g., schizophrenia, depression) were associated with perceptions that people with these disorders are dangerous and violent (Hartini et al., 2018; Parcesepe & Cabassa, 2013). People diagnosed with grief-related mental disorders might be at greater risk of being stigmatized. For example, results of a recent cross-sectional study in Portugal showed that grief-related mental disorders were less recognized when compared to other mental health conditions (e.g., depression; Cunha, 2021). Examining the public stigma directed at grief experiences is important not only because public stigma has been associated with reduced help-seeking behavior (Pattyn et al., 2014; Pedersen & Paves, 2014), but can also predict the development of self-stigma later on (Vogel et al., 2013).

Similar to other mental disorders, people diagnosed with PGD have an increased risk of being stigmatized,

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when compared to people who report no symptoms and are more adapted to the loss experience (i.e., integrated grief; Gonschor et al., 2020). Indeed, past research has shown that targets described as being diagnosed with PGD elicited more negative emotions (i.e., more anger, prosocial reactions, and fear), and were more negatively appraised (i.e., perceived as less competent, warm, and emotionally stable, and as more dependent and sensitive; Eisma, 2018; Eisma et al., 2019; Gonschor et al., 2020). However, some of these findings have not been consistently replicated in more recent studies (e.g., lack of significant effects on anger, perceived warmth, dependence, or sensitivity; see Dennis et al., 2022; Zammit et al., 2023). Importantly, targets in all these studies were described as having lost a romantic partner or a friend and research has yet to determine if public stigma extends to other types of relationship losses. Grief experiences that are not socially supported, acknowledged, or perceived as legitimate tend to be perceived as less valid (i.e., disenfranchized grief; Corr, 1999) and have an increased risk of being stigmatized. For example, researchers have argued that public stigma is likely to increase for bereaved people with grief symptoms after the loss of a companion animal (i.e., disenfranchized grief), as this experience tends to be socially overlooked and stigmatized by others (Crossley & Rolland, 2022). Indeed, Brown et al. (2023) found that one in three bereaved participants felt that their grief caused by the loss of a companion animal was not recognized and supported by others. Likewise, Cesur-Soysal and Arı (2022) found that people were less likely to offer social support to someone who lost a companion animal when compared to going through a romantic relationship break-up or having a psychological disorder. As a result, bereaved people who lost their companion animal tend to avoid sharing their experiences with others or fail to receive the social support they need (Packman et al., 2014). And when bereaved people feel socially isolated or unable to share their feelings, they are more likely to question their own mental health (Cleary et al., 2022), and potentially experience downstream consequences in their well-being.

Current study and hypotheses

Replicating and extending recent studies (Dennis et al., 2022; Eisma, 2018; Eisma et al., 2019; Gonschor et al., 2020; Zammit et al., 2023), we conducted a vignette-based experimental study to examine the stigmatization of targets who lost their romantic partner, child, and companion animal, depending on their

grief experience. Building upon past findings, we expected targets with PGD diagnosis (*vs.* integrated grief) to elicit more negative emotional reactions (i.e., more anger, prosocial reactions, and fear; H1a) and more negative attributions (i.e., perceived as less competent, warm, and emotionally stable; perceived as more dependent and sensitive; H1b). We also expected these targets to have their grief experiences perceived as less legitimate (H1c).

Given that experiencing severe grief symptoms after the loss of a companion animal increases the risk of public stigma (Crossley & Rolland, 2022), we also expected targets who lost their companion animal (*vs.* romantic partner; *vs.* child) to elicit more negative emotional reactions (H2a), more negative attributions (H2b), and their grief experiences to be perceived as less legitimate (H2c).

Lastly, we explored if the type of relationship loss changed the public stigma associated with having a PGD diagnosis. Specifically, we expected a stronger public stigma (i.e., more negative emotional reactions [H3a], more negative attributions [H3b], and less perceived legitimacy [H3c]) for targets with PGD diagnosis who lost their companion animal (*vs.* romantic partner; *vs.* child). No differences according to the type of relationship loss were expected for targets with integrated grief.

Past studies have shown mixed findings regarding the effect of a target's gender on public stigma indicators. For example, Dennis et al. (2022) found no differences between male and female targets, whereas Gonschor et al. (2020) found that a male (*vs.* female) target elicited higher preferred social distance. Based on this, we decided to manipulate the target's gender in our design but not to offer *a priori* hypotheses. Instead, we explored if public stigma indicators changed depending on whether the target was male or female (or if this factor interacted with the other factors of our experimental design).

Methods

Participants and design

Based on the lowest effect size reported in past research ($\eta_p^2 = .030$; Eisma et al., 2019), a power analysis using G*Power (Faul et al., 2009) indicated that we needed 196 participants to detect a significant effect in a repeated-measures mixed ANOVA with 95% power. We increased this estimation by 20% to account for participants who had to be removed, resulting in a target sample of 235 participants.

From the 345 prospective participants who accessed the survey, we removed people who failed to give their consent ($n=3$) or complete the survey ($n=33$), indicated to have paid no attention or very little attention to the survey ($n=2$), and wanted their responses withdrawn ($n=1$). The final sample included 306 participants who were, on average, 36 years old. Demographic information for the entire sample and each experimental condition is summarized in Table 1. Overall, most participants identified as White, completed more than 12 years of education, and were working at either a part-time or a full-time job. Half of our participants identified as female, were living in a metropolitan area, had been coping with their present income, and were either married or in a *de facto* union.

Participants were randomly assigned to one of the experimental conditions in a 2 (grief experiences: PGD diagnosis *vs.* integrated grief) $\times$ 2 (target's gender: female *vs.* male) $\times$ 3 (type of relationship loss: romantic partner *vs.* child *vs.* companion animal), with the latter factor as within-participants.

Materials

We developed 12 vignettes based on past research (Dennis et al., 2022; Eisma et al., 2019; Gonschor et al., 2020). Participants were presented with three vignettes, each describing a different relationship loss (i.e., romantic partner, child, and companion animal). Targets were given different names, selected based on the most popular Portuguese names to avoid

Table 1. Sample characteristics of the total sample ($N=306$) and by experimental group.

	Total sample ($N=306$)	PGD diagnosis female ($n=78$)	PGD diagnosis male ($n=76$)	Integrated grief female ($n=79$)	Integrated grief male ($n=73$)
	<i>M</i> (<i>SD</i>) or <i>N</i> (%)	<i>M</i> (<i>SD</i>) or <i>N</i> (%)	<i>M</i> (<i>SD</i>) or <i>N</i> (%)	<i>M</i> (<i>SD</i>) or <i>N</i> (%)	<i>M</i> (<i>SD</i>) or <i>N</i> (%)
Age	35.88 (9.98)	35.28 (10.15)	35.78 (9.16)	36.18 (10.54)	36.33 (10.17)
Gender					
Female	158 (51.6)	37 (47.4)	46 (60.5)	35 (44.3)	40 (54.8)
Male	146 (47.8)	41 (52.6)	30 (39.5)	44 (55.7)	31 (42.5)
Non-binary	1 (0.3)	–	–	–	1 (1.4)
Prefer not to answer	1 (0.3)	–	–	–	1 (1.4)
Ethnicity					
Asian	1 (0.3)	–	–	1 (1.3)	–
Black	22 (7.2)	4 (5.1)	3 (3.9)	5 (6.3)	10 (13.7)
Latin	34 (11.1)	5 (6.4)	12 (15.8)	11 (13.9)	6 (8.2)
White	240 (78.4)	67 (85.9)	58 (76.3)	60 (75.9)	55 (75.3)
Other (e.g., Sephardi)	4 (1.3)	1 (1.3)	2 (2.6)	–	1 (1.4)
Prefer not to answer	3 (1.0)	1 (1.3)	–	2 (2.5)	–
Education level					
Less than or equal to 12 years	101 (33.1)	28 (35.9)	19 (25.0)	21 (26.6)	33 (45.2)
More than 12 years	200 (65.4)	49 (62.8)	56 (73.7)	58 (73.4)	37 (50.7)
Prefer not to answer	3 (1.0)	1 (1.3)	–	–	2 (2.7)
Employment status					
Student	40 (13.1)	10 (12.8)	11 (14.4)	11 (13.9)	8 (10.9)
Working (part-time or full-time)	230 (75.1)	60 (76.9)	53 (69.7)	60 (76.0)	56 (76.7)
Stay-at-home parent	4 (1.3)	1 (1.3)	1 (1.3)	1 (1.3)	1 (1.4)
Unemployed	27 (8.8)	7 (9.0)	8 (10.5)	7 (8.9)	5 (6.9)
Retired	1 (0.3)	–	–	–	1 (1.4)
Prefer not to answer	2 (0.7)	–	1 (1.3)	–	1 (1.4)
Location of residency					
Metropolitan area	167 (54.6)	40 (51.3)	38 (50.0)	40 (50.6)	49 (67.1)
Peripheral cities	91 (29.7)	21 (26.9)	25 (32.9)	28 (35.4)	17 (23.3)
Rural area	45 (14.8)	17 (21.8)	12 (15.8)	10 (12.7)	6 (8.2)
Prefer not to answer	1 (0.3)	–	–	1 (1.3)	–
Perceived socioeconomic status					
Having difficulty with present income	96 (31.4)	26 (33.3)	24 (31.6)	23 (29.1)	23 (31.5)
Coping with present income	151 (49.3)	35 (44.9)	37 (48.7)	42 (53.2)	37 (50.7)
Living comfortably with present income	52 (17.0)	16 (20.5)	13 (17.1)	13 (16.5)	10 (13.7)
Prefer not to answer	5 (1.6)	1 (1.3)	1 (1.3)	1 (1.3)	2 (2.7)
Relationship status					
No significant romantic relationship	51 (16.7)	12 (15.4)	11 (14.5)	15 (19.0)	13 (17.8)
Casual relationship(s) with one or more people	19 (6.2)	6 (7.7)	4 (5.3)	3 (3.8)	6 (8.2)
Significant relationship(s) with one or more people	75 (24.5)	15 (19.2)	17 (22.4)	22 (27.8)	21 (28.8)
Married or in a <i>de facto</i> union	151 (49.3)	43 (55.1)	41 (53.9)	36 (45.6)	31 (42.5)
Other (e.g., separated)	5 (1.6)	1 (1.3)	2 (2.6)	2 (2.5)	–
Prefer not to answer	3 (1.0)	1 (1.3)	–	1 (1.3)	1 (1.4)

PGD: prolonged grief disorder.

idiosyncrasies or automatic inferences. Compatible with the DSM-5-TR (Prigerson et al., 2021), vignettes in the PGD diagnosis conditions included two criteria—experiencing grief symptoms for more than 12 months and having significant impairments in functioning—and five symptoms—longing for the deceased, sense of disbelief, avoidance of reminders, feeling that life is meaningless, and experiencing difficulties with reintegration into life. Targets were described as being 50 years old and having experienced the symptoms for more than two years (see Dennis et al., 2022; Eisma et al., 2019). Full descriptions used in each vignette are presented in Table 2.

Dependent measures

Emotional reactions

We used the 13-item Emotional Reaction Scale adapted by Eisma et al. (2019) to assess anger (four items; e.g., “I feel annoyed”), prosocial reactions (four items; e.g., “I feel the need to help”), and fear (five items; e.g., “I feel nervous”) toward the targets. Responses to each item were given on 7-point rating scales (1 = *Strongly disagree* to 7 = *Strongly agree*). Responses to each subscale were mean averaged, with higher scores indicating more anger ($\alpha = .78$), prosocial reactions ($\alpha = .84$), and fear ($\alpha = .88$).

Attribution of traits

We used the 5-item attribution measure proposed by Eisma (2018) to assess the degree to which targets were perceived as competent, warm, emotionally stable, dependent, and sensitive. Responses to each item

were given on 7-point rating scales (1 = *Strongly disagree* to 7 = *Strongly agree*). We used raw scores, with higher scores indicating stronger attributions.

Legitimacy of grief experiences

We developed a 3-item measure to assess the extent to which participants perceived the grief experienced by each target as legitimate, appropriate, and adjusted. Responses to each item were given on 7-point rating scales (1 = *Not at all* to 7 = *Very much*). We computed a mean score by averaging responses ($\alpha = .71$), with higher scores indicating more perceived legitimacy of grief experiences.

Procedure

This study was approved by the Ethics Council at Iscte-Instituto Universitário de Lisboa beforehand (#22/2021). Prospective participants were recruited on the Clickworker platform and data was collected on a survey hosted on Qualtrics. To be eligible to participate in this study, participants had to be over the age of 18, reside in Portugal, and speak Portuguese fluently. The informed consent provided a detailed description of the main aims of the study, the estimated time of completion, the potential distress participants might experience while reading grief-related scenarios, the reward they would be credited with upon their participation, and their rights to abandon the experiment without providing any justification. After providing consent, participants were presented with the standard demographic information form (e.g., age, gender, education). Next, participants were

Table 2. Vignettes in the study.

Vignettes	Descriptions
PGD diagnosis/female character	Fifty-year old [<i>Maria/Ana/Sofia</i>] has lost her [<i>husband/child/companion animal</i>] to a stroke more than two years ago. She finds this extremely difficult and does not function well at work nor at home. Since the loss, she yearns strongly for her deceased [<i>husband/child/companion animal</i>]. Additionally, she has difficulties accepting the loss, does not want to be reminded of the loss, finds her life meaningless and has difficulties trusting others. On the basis of this behavior, a mental health professional diagnosed her with prolonged grief disorder.
PGD diagnosis/male character	Fifty-year old [<i>João/António/Nuno</i>] has lost his [<i>wife/child/companion animal</i>] to a stroke more than two years ago. He finds this extremely difficult and does not function well at work nor at home. Since the loss, he yearns strongly for his deceased [<i>wife/child/companion animal</i>]. Additionally, he has difficulties accepting the loss, does not want to be reminded of the loss, finds his life meaningless and has difficulties trusting others. On the basis of this behavior, a mental health professional diagnosed him with prolonged grief disorder.
Integrated grief/female character	Fifty-year old [<i>Maria/Ana/Sofia</i>] has lost her [<i>husband/child/companion animal</i>] to a stroke more than two years ago. While she was very sad the first few months after the loss, she now has managed to cope with the loss. She functions well both at work and at home. [<i>Maria/Ana/Sofia</i>] has accepted the loss of her [<i>husband/child/companion animal</i>] more, occasionally engages in fond reminiscences of her [<i>husband/child/companion animal</i>], and feels her life is meaningful.
Integrated grief/male character	Fifty-year old [<i>João/António/Nuno</i>] has lost his [<i>wife/child/companion animal</i>] to a stroke more than two years ago. While he was very sad the first few months after the loss, he now has managed to cope with the loss. He functions well both at work and at home. [<i>João/António/Nuno</i>] has accepted the loss of his [<i>wife/child/companion animal</i>] more, occasionally engages in fond reminiscences of his [<i>wife/child/companion animal</i>], and feels his life is meaningful.

PGD: prolonged grief disorder.

Note. Manipulations for the type of relationship loss (i.e., within-participants conditions) are represented in italics. In total, there were 12 vignettes.

randomly allocated to one of the experimental conditions and presented with three scenarios (one for each type of relationship loss) in random order. For each scenario, participants were asked to complete the dependent measures. After completing this task, participants were presented with the manipulation checks (Dennis et al., 2022; Eisma et al., 2019; Gonschor et al., 2020) and asked: “Were the people described in the stories diagnosed with prolonged grief disorder?” (1 = *Yes*, 2 = *No*, 3 = *I don’t know*) and “What was the sex of the people described in the scenarios?” (1 = *Male*, 2 = *Female*, 3 = *I don’t know*).

As control measures, participants were additionally asked if they owned a companion animal (1 = *Yes*, 2 = *No*) and if they experienced any loss in the past (1 = *Yes*, 2 = *No*, 3 = *Prefer not to answer*). Before ending the survey, participants were asked “How much attention did you pay to the questionnaire while filling it out?” (1 = *No attention at all*, 2 = *Very little attention*, 3 = *A moderate amount of attention*, 4 = *Very much attention*) and asked to indicate if they wanted to retain their answers for analysis (1 = *I want to keep my responses for analysis*, 2 = *I want to remove my responses and prevent them from being analyzed*). In the end, participants were thanked, informed about the aim of the study, and provided with the contact information of the researchers. Debriefing also included contact information for public health services, in case participants considered they felt distressed or anxious after participating, or wanted to learn more about how to access professional mental health services in Portugal. Upon successfully submitting their completed survey, participants received 1.5€ on their Clickworker account.

Data analytic plan

We started by running assumptions, randomization, and manipulation checks. Our hypotheses were then tested in a 2 (grief experiences: PGD diagnosis vs. integrated grief) $\times$ 2 (target’s gender: female vs. male) $\times$ 3 (type of relationship loss: romantic partner vs. child vs. companion animal), with the latter factor as within-participants. We conducted repeated measures ANOVAs for each dependent variable. When significant interaction or interactions were found, we computed post-hoc comparisons with Bonferroni adjustment. Given that most participants indicated they owned a companion animal (89.2%) and experienced a loss in the past (93.5%), we were unable to explore if our results changed when these variables were controlled for.

Results

Assumption checks

We started by checking sphericity in our data using Mauchly’s test. Results showed sphericity in two of our dependent variables (i.e., emotionally stable and sensitive), both $p \geq .051$. For the remaining dependent measures, results showed violations of sphericity, all $p \leq .017$, and therefore we applied the Huynh-Feldt correction in these cases.

Randomization checks

Results showed no significant differences between the four groups based on age, $F(3, 302) = 0.17$, $p = .918$, gender, $\chi^2(12) = 0.12$, $p = .282$, ethnicity, $\chi^2(15) = 0.14$, $p = .202$, education, $\chi^2(18) = 0.16$, $p = .211$, employment status, $\chi^2(21) = 0.12$, $p = .874$, location of residency, $\chi^2(12) = 0.14$, $p = .150$, perceived socioeconomic status, $\chi^2(9) = 0.53$, $p = .979$, or relationship status, $\chi^2(15) = 0.10$, $p = .877$.

Manipulation checks

Most participants correctly indicated that targets did not experience any grief-related symptoms or receive a diagnosis (i.e., integrated grief) in the respective experimental condition (70.4%) and that targets received a PGD diagnosis in the respective experimental condition (92.2%). Similarly, most participants correctly indicated the sex of the targets in both female (96.2%) and male conditions (98.6%). We conducted the main analyses with the entire sample and after excluding participants who failed to give the correct answer in the manipulation checks ($n = 65$). The pattern of results was similar in both cases; therefore, we reported only the results for the total sample.

Main analyses

The main effects and interactions are summarized in Table 3. We found no main effect of the target’s gender, nor interactions with the other experimental factors, indicating that perceptions were similar for female and male bereaved targets.² The only exception was a 3-way interaction for anger, indicating that male targets with PGD diagnosis (vs. integrated grief) who lost their child elicited less anger, $p = .048$, and that female and male targets with PGD diagnosis (vs. integrated grief) who lost their companion animal elicited more anger, both $p < .001$.

Table 3. Main effects and interactions in all dependent variables.

	Main effects			Interactions			
	Grief experiences	Type of relationship loss	Target's gender	Grief experiences * Type of relationship loss	Grief experiences * Target's gender	Type of relationship loss * Target's gender	3-way
Emotional reactions							
Anger	$F(1, 302)=4.15$, $p=.042$, $\eta^2=.010$	$F(1.89, 571.10)=1.46$, $p=.235$, $\eta^2=.001$	$F(1, 302)=1.13$, $p=.288$, $\eta^2=.003$	$F(1.89, 571.10)=24.25$, $p<.001$, $\eta^2=.023$	$F(1, 302)=1.19$, $p=.276$, $\eta^2=.003$	$F(1.89, 571.10)=2.30$, $p=.104$, $\eta^2=.002$	$F(1.89, 571.10)=3.12$, $p=.048$, $\eta^2=.003$
Prosocial reactions	$F(1, 302)=71.09$, $p<.001$, $\eta^2=.145$	$F(1.81, 548.09)=86.76$, $p<.001$, $\eta^2=.075$	$F(1, 302)=0.43$, $p=.513$, $\eta^2=.001$	$F(1.81, 548.09)=3.46$, $p=.037$, $\eta^2=.003$	$F(1, 302)=0.01$, $p=.901$, $\eta^2=.000$	$F(1.81, 548.09)=2.86$, $p=.063$, $\eta^2=.003$	$F(1.81, 548.09)=0.09$, $p=.901$, $\eta^2=.000$
Fear	$F(1, 302)=10.61$, $p=.001$, $\eta^2=.027$	$F(1.97, 593.92)=17.19$, $p<.001$, $\eta^2=.011$	$F(1, 302)=0.66$, $p=.417$, $\eta^2=.002$	$F(1.97, 593.92)=4.59$, $p=.011$, $\eta^2=.003$	$F(1, 302)=0.45$, $p=.503$, $\eta^2=.001$	$F(1.97, 593.92)=1.65$, $p=.193$, $\eta^2=.001$	$F(1.97, 593.92)=.581$, $p=.054$, $\eta^2=.000$
Attributions							
Competent	$F(1, 302)=66.01$, $p<.001$, $\eta^2=.133$	$F(1.86, 563.04)=1.68$, $p=.190$, $\eta^2=.002$	$F(1, 302)=2.43$, $p=.120$, $\eta^2=.006$	$F(1.86, 563.04)=8.94$, $p<.001$, $\eta^2=.009$	$F(1, 302)=0.60$, $p=.439$, $\eta^2=.001$	$F(1.86, 563.04)=0.83$, $p=.429$, $\eta^2=.001$	$F(1.86, 563.04)=1.08$, $p=.337$, $\eta^2=.001$
Warm	$F(1, 302)=4.44$, $p=.036$, $\eta^2=.011$	$F(1.98, 597.85)=5.92$, $p=.003$, $\eta^2=.005$	$F(1, 302)=0.27$, $p=.606$, $\eta^2=.001$	$F(1.98, 597.85)=3.18$, $p=.043$, $\eta^2=.003$	$F(1, 302)=0.02$, $p=.883$, $\eta^2=.000$	$F(1.98, 597.85)=0.16$, $p=.854$, $\eta^2=.000$	$F(1.98, 597.85)=1.02$, $p=.362$, $\eta^2=.001$
Emotionally stable	$F(1, 301)=203.12$, $p<.001$, $\eta^2=.321$	$F(2, 602)=1.17$, $p=.311$, $\eta^2=.001$	$F(1, 301)=1.78$, $p=.183$, $\eta^2=.004$	$F(2, 602)=4.17$, $p=.016$, $\eta^2=.004$	$F(1, 301)=0.20$, $p=.658$, $\eta^2=.000$	$F(2, 602)=1.46$, $p=.233$, $\eta^2=.001$	$F(2, 602)=1.21$, $p=.300$, $\eta^2=.001$
Dependent	$F(1, 302)=63.48$, $p<.001$, $\eta^2=.128$	$F(1.90, 574.36)=0.18$, $p=.827$, $\eta^2=.000$	$F(1, 302)=0.72$, $p=.397$, $\eta^2=.002$	$F(1.90, 574.36)=2.15$, $p=.120$, $\eta^2=.002$	$F(1, 302)=0.24$, $p=.626$, $\eta^2=.001$	$F(1.90, 574.36)=0.14$, $p=.864$, $\eta^2=.000$	$F(1.90, 574.36)=0.34$, $p=.701$, $\eta^2=.000$
Sensitive	$F(1, 301)=55.17$, $p<.001$, $\eta^2=.115$	$F(2, 602)=4.93$, $p=.008$, $\eta^2=.005$	$F(1, 301)=0.00$, $p=.961$, $\eta^2=.000$	$F(2, 602)=9.85$, $p<.001$, $\eta^2=.009$	$F(1, 301)=0.51$, $p=.474$, $\eta^2=.001$	$F(2, 602)=2.80$, $p=.062$, $\eta^2=.003$	$F(2, 602)=0.38$, $p=.681$, $\eta^2=.000$
Legitimacy	$F(1, 302)=14.86$, $p<.001$, $\eta^2=.032$	$F(1.65, 497.42)=31.60$, $p<.001$, $\eta^2=.034$	$F(1, 302)=1.74$, $p=.189$, $\eta^2=.004$	$F(1.65, 497.42)=71.00$, $p<.001$, $\eta^2=.073$	$F(1.65, 497.42)=2.86$, $p=.092$, $\eta^2=.006$	$F(1.65, 497.42)=1.81$, $p=.171$, $\eta^2=.002$	$F(1.65, 497.42)=0.29$, $p=.702$, $\eta^2=.000$

We found main effects of grief experiences on all dependent measures. As expected, targets with PGD diagnosis (vs. integrated grief) elicited more anger ($M=2.10$, $SD=0.93$ vs. $M=1.88$, $SD=0.96$), more prosocial reactions ($M=4.99$, $SD=1.12$ vs. $M=3.91$, $SD=1.10$), and more fear ($M=2.24$, $SD=1.08$ vs. $M=1.84$, $SD=1.04$), were perceived as less competent ($M=4.20$, $SD=1.35$ vs. $M=5.40$, $SD=1.24$), less warm ($M=4.52$, $SD=1.57$ vs. $M=4.88$, $SD=1.37$), less emotionally stable ($M=3.17$, $SD=1.53$ vs. $M=5.45$, $SD=1.26$), and were perceived as more dependent ($M=4.33$, $SD=1.39$ vs. $M=2.99$, $SD=1.54$), and more sensitive ($M=5.85$, $SD=1.15$ vs. $M=4.80$, $SD=1.30$). Lastly, participants perceived the grief of targets with PGD diagnosis (vs. integrated grief) as less legitimate ($M=4.94$, $SD=1.11$ vs. $M=5.44$, $SD=1.14$).

We also found main effects of the type of relationship loss in five dependent variables (no other main effects emerged, $p\geq.190$). As expected, targets who lost their companion animal (vs. romantic

partner; vs. child) were perceived as the most sensitive, both $p\leq.033$, and their grief experiences were perceived as the least legitimate, both $p<.001$. Against our expectations, targets who lost their companion animal (vs. romantic partner; vs. child) elicited the lowest prosocial reactions, both $p<.001$, and the least fear, both $p<.001$, and were perceived as the warmest, both $p\leq.036$. No differences between targets who lost their romantic partner or child were found, all $p\geq.078$ (see Table 4).

Significant interactions between grief experiences and the type of relationship loss emerged in eight dependent variables (no other interactions were found, $p\geq.120$). As shown in Table 5, targets with PGD diagnosis (vs. integrated grief) elicited overall more negative emotional reactions, more negative attributions, and less perceived legitimacy across all types of relationship losses, with larger differences observed for targets who lost their companion animal. As expected, targets with PGD diagnosis who lost their companion

Table 4. Descriptive statistics for emotional reactions, attributions, and legitimacy according to type of relationship loss.

	Type of relationship loss		
	Romantic partner	Child	Companion animal
	<i>M (SD)</i>	<i>M (SD)</i>	<i>M (SD)</i>
Emotional reactions			
Anger	1.93 ^a (1.04)	2.04 ^a (1.16)	2.00 ^a (1.21)
Prosocial reactions	4.53 ^a (1.38)	4.85 ^b (1.35)	3.96 ^c (1.51)
Fear	2.08 ^a (1.27)	2.17 ^a (1.23)	1.87 ^b (1.09)
Attributions			
Competent	4.83 ^a (1.59)	4.86 ^a (1.63)	4.72 ^a (1.77)
Warm	4.64 ^a (1.69)	4.60 ^a (1.73)	4.87 ^b (1.69)
Emotionally stable	4.37 ^a (2.00)	4.24 ^a (2.05)	4.32 ^a (2.04)
Dependent	3.68 ^a (1.93)	3.63 ^a (1.81)	3.68 ^a (1.90)
Sensitive	5.25 ^a (1.52)	5.25 ^a (1.64)	5.46 ^b (1.50)
Legitimacy	5.38 ^a (1.22)	5.36 ^a (1.37)	4.83 ^b (1.70)

Note. Different superscripts ^(a,b) indicate significant differences with Bonferroni correction, all $p \leq .050$.

animal (vs. romantic partner; vs. child) elicited the highest anger reactions, both $p < .001$, were perceived as the least competent, both $p \leq .002$, and their grief experiences were perceived as the least legitimate, both $p < .001$. Unexpectedly, these targets also elicited the lowest prosocial reactions, both $p < .001$. No other differences reached significance, all $p \geq .117$. Also unexpectedly, targets with integrated grief who lost their companion animal (vs. romantic partner; vs. child) elicited the lowest prosocial, both $p < .001$, and fear reactions, both $p \leq .003$, were perceived as the warmest, both $p \leq .007$, and the most sensitive, both $p < .001$. No other significant differences between targets with integrated grief emerged across three types of relationship losses, all $p \geq .054$.

Discussion

Building upon recent studies (Dennis et al., 2022; Eisma, 2018; Eisma et al., 2019; Gonschor et al., 2020; Zammit et al., 2023), our study was the first to experimentally compare public stigma indicators toward bereaved people with different relationship losses (e.g., romantic partner, child, companion animal) in a sample of Portuguese residents who were fluent in Portuguese. We also explored whether grief experiences typically perceived as less valid (i.e., disenfranchised grief; Corr, 1999) are more publicly stigmatized. We decided to exclude the social distance measure used in previous research (Dennis et al., 2022; Eisma, 2018; Eisma et al., 2019; Gonschor et al., 2020; Zammit et al., 2023) and instead assessed the perceived legitimacy of grief experienced by each target. From our perspective, this measure offered novel insights to understand if some relationship losses are

more (or less) likely to be stigmatized and potentially help explain the roots of this public stigma phenomenon.

Complementing past research (Dennis et al., 2022), we found no overall main effects or interactions of the target's gender. These results indicate that public stigma toward grieving people is largely independent of whether the bereaved person is a woman or a man. Consistent with our hypotheses, we found that targets with PGD diagnosis (vs. integrated grief) elicited more negative emotional reactions (e.g., anger, fear; H1a) and were perceived as having more negative traits (e.g., less competent, less emotionally stable; H1b) when compared to targets without such symptoms and diagnosis (i.e., with integrated grief). These results indicate the existence of a public stigma toward people who are coping with severe grief symptoms and support the cross-cultural generalizability of the phenomenon. Extending past research, we showed for the first time that the grief experiences of targets with PGD diagnosis (vs. integrated grief) were perceived as less legitimate (H1c). This indicates that people coping with a loss—and particularly those with severe symptoms and a clinical diagnosis—are more likely to be misunderstood by others and have their grief experiences minimized or overlooked by them. Resulting of the social stigma directed at them, people may refrain from seeking help from informal (e.g., from members of the close network) and formal support systems (e.g., from a mental health professional; Pattyn et al., 2014). When experiencing more severe grief symptoms, lacking support and recognition from others can increase the risk of negative health outcomes later on.

Targets who lost their companion animal (vs. their romantic partner or child) were perceived as more sensitive (partially supporting H2b) and their grief as less legitimate (supporting H2c). Notably, these results emerged even when most of our participants reported having a companion animal and having experienced a loss (~90%), thus indicating the robustness of the public stigma toward disenfranchised grief (i.e., loss of a companion animal). Unexpectedly, however, these targets also elicited less prosocial and fear reactions (against H2a) and were perceived as warmer (partially against H2b). These findings showed for the first time some degree of public stigma toward specific relationship losses. Our results also complement past evidence that society is less likely to offer social support when (or if) needed (Brown et al., 2023; Cesur-Soysal & Ari, 2022) and to acknowledge the loss of a companion animal as a legitimate experience (at least compared to the loss of a close person). And yet, this

Table 5. Descriptive statistics for emotional reactions, attributions, and legitimacy according to grief experiences and type of relationship loss.

	Type of relationship loss: Romantic partner		Type of relationship loss: Child		Type of relationship loss: Companion animal	
	Integrated grief	PGD diagnosis	Integrated grief	PGD diagnosis	Integrated grief	PGD diagnosis
	<i>M</i> (<i>SD</i>)	<i>M</i> (<i>SD</i>)	<i>M</i> (<i>SD</i>)	<i>M</i> (<i>SD</i>)	<i>M</i> (<i>SD</i>)	<i>M</i> (<i>SD</i>)
Emotional reactions						
Anger	1.85 ^{a,1} (1.08)	2.02 ^{a,1} (1.01)	2.11 ^{a,2} (1.30)	1.95 ^{a,1} (1.00)	1.65 ^{a,1} (0.97)	2.32 ^{b,2} (1.33)
Prosocial reactions	3.89 ^{a,1} (1.28)	5.16 ^{b,1} (1.19)	4.33 ^{a,2} (1.25)	5.38 ^{b,2} (1.24)	3.51 ^{a,3} (1.39)	4.41 ^{b,3} (1.50)
Fear	1.86 ^{a,1} (1.22)	2.29 ^{b,1} (1.30)	2.06 ^{a,2} (1.23)	2.29 ^{a,1} (1.23)	1.60 ^{a,3} (1.01)	2.14 ^{b,1} (1.11)
Attributions						
Competent	5.36 ^{a,1} (1.45)	4.32 ^{b,1} (1.55)	5.35 ^{a,1} (1.51)	4.38 ^{b,1} (1.60)	5.53 ^{a,1} (1.42)	3.92 ^{b,2} (1.71)
Warm	4.78 ^{a,1} (1.58)	4.51 ^{a,1} (1.79)	4.70 ^{a,1} (1.63)	4.50 ^{a,1} (1.81)	5.16 ^{a,2} (1.50)	4.57 ^{b,1} (1.81)
Emotionally stable	5.47 ^{a,1} (1.46)	3.29 ^{b,1} (1.85)	5.30 ^{a,1} (1.64)	3.18 ^{b,1} (1.87)	5.62 ^{a,2} (1.41)	3.03 ^{b,1} (1.73)
Dependent	2.98 ^{a,1} (1.85)	4.39 ^{b,1} (1.75)	3.07 ^{a,1} (1.80)	4.19 ^{b,1} (1.64)	2.93 ^{a,1} (1.74)	4.42 ^{b,1} (1.77)
Sensitive	4.66 ^{a,1} (1.56)	5.84 ^{b,1} (1.22)	4.60 ^{a,1} (1.67)	5.89 ^{b,1} (1.34)	5.14 ^{a,2} (1.44)	5.80 ^{b,1} (1.48)
Legitimacy	5.46 ^{a,1} (1.28)	5.32 ^{a,1} (1.16)	5.26 ^{a,1} (1.46)	5.46 ^{a,1} (1.27)	5.62 ^{a,2} (1.33)	4.06 ^{b,2} (1.67)

PGD: prolonged grief disorder.

Note. Different superscripts indicate significant differences within companion animal loss according to PGD diagnosis and integrated grief (^{a,b}) and within three types of relationship losses according to PGD diagnosis and integrated grief (^{1,2}) with Bonferroni correction, all $p \leq .040$.

type of relationship loss also elicited less negative emotional reactions and attributions (but only in certain dependent measures), contrasting with our original expectations. Although we were unable to replicate some of the results from past studies (see also Dennis et al., 2022; Zammit et al., 2023), we believe that perceived legitimacy can be valuable in future studies to explain some of our apparent contradictory findings.

Furthermore, we found that targets with PGD diagnosis who lost their companion animal (vs. romantic partner or child) elicited the strongest negative emotional reactions (partially supporting H3a), were perceived the most negatively (supporting H3b), and their grief was perceived as the least legitimate (supporting H3c). These findings suggest that in comparison with other types of relationship losses (i.e., losing a close person), experiencing severe grief symptoms after the loss of a companion animal elicited more public stigma. In line with the public stigma hypotheses, targets with PGD diagnosis (vs. integrated grief) who lost their companion animal elicited overall stronger negative emotional reactions, were perceived more negatively, and their grief was perceived as less legitimate. Possibly, these targets were subject to a double stigma (i.e., severe grief symptoms *and* disenfranchized grief). These findings suggest that social norms and expectations surrounding the loss of a close person differ from those surrounding the loss of a companion animal. Such social inequalities may lead to fewer intentions and being less motivated to seek out social support after the loss of a companion animal (Crossley & Rolland, 2022). Additionally, the lack of social validation and acknowledgment of the grief after the loss of a companion animal (Wong et al., 2017), insufficient public policies that support bereaved people to take

time off from work to cope with their loss (Cowling et al., 2020), and lower perceived social support from close others (Sarper & Rodrigues, 2024) may contribute to the experience of more severe grief symptoms later on. Contrasting with these findings, targets with integrated grief after the loss of their companion animal (vs. romantic partner or child) elicited more positive emotional reactions (i.e., less prosocial and fear reactions), and more positive attributions (i.e., warmer). Considering previous research (including our own study) showing public stigma toward bereaved people experiencing disenfranchized grief (e.g., Crossley & Rolland, 2022), our findings suggest that being able to integrate grief after the loss of a companion animal can decrease the likelihood of experiencing public stigma (or at least, people in this situation are merely perceived as having an adjusted reaction given the type of this relationship loss).

Limitations and future research

Our study had limitations that must be discussed. First, the use of a within-participants design in the relationship loss scenarios might have facilitated comparisons and increased the likelihood of order effects in our data (e.g., carry-over effects, practice, fatigue). However, we believe that randomly presenting the scenarios helped decrease such possibility and bolstered our confidence in the results herein reported. Nonetheless, future studies could seek to replicate our study using a full between-participants design. Second, our experimental materials closely followed those used in past research (Dennis et al., 2022; Eisma, 2018; Eisma et al., 2019) and presented 50 year old targets. Future research could seek to examine whether public

stigma toward PGD and disenfranchised grief differs depending on the target's age. For example, past research has shown a positive association between age and mindfulness components, such as present-moment attention and nonjudgmental orientation (Mahlo & Windsor, 2021). Arguably, younger adults may be perceived as less able to cope with stressful life events (e.g., losing a close other or a companion animal) and people may be more understanding and empathetic for their experiences, and possibly, less stigmatizing.

Our study offered causal evidence regarding the increased risk of public stigma toward bereaved people who lost a companion animal, particularly those who experience more severe grief symptoms. However, we did not examine whether this stigma translates into real-world differences in mental health (e.g., lower self-esteem) or worse social outcomes (e.g., lack of supportive social networks) among bereaved people. Future studies could seek to examine the stigma experienced from the perspective of grieving people, its implications for health and well-being over time, and how social support received from close others can help cope with perceived stigma.

Lastly, we developed our materials informed by past research although some of our experimental conditions may lack ecological validity. Based on the DSM-5-TR (Prigerson et al., 2021), bereaved people can only be diagnosed with PGD after the loss of a person. From our perspective, discussions surrounding the validity of a PGD diagnosis after the loss of a companion animal (as described in our vignette) are mostly held by academics and professionals, and people are likely to be unaware of this detail. Hence, we decided to have this particular vignette for two reasons. First, research has shown that bereaved people who lost a companion animal can experience grief symptoms similar to those experienced by people who lost a close person (Lee, 2020). Second, we wanted to have equivalent contexts in the PGD conditions, to statistically compare public stigma indicators. We believe that our methodological choice had no negative impact on our study and that our results added to the discussion of whether the criteria for PGD diagnosis should be revised to become more inclusive.

Implications

Past studies have shown that people can experience comparable levels of grief severity after the loss of a close person or a companion animal (Lavorgna &

Hutton, 2019). And yet, people who lost a companion animal and cope with severe grief symptoms are less likely to have their loss acknowledged and valued (Spain et al., 2019), or perceived as a legitimate experience by others. As such, bereaved people who lost their companion animals have an increased risk of perceiving lower social support from close others (Brown et al., 2023). Based on the extensive evidence showing that social support has benefits for health and well-being outcomes (Siedlecki et al., 2014; Wilson et al., 2020) and can buffer negative experiences during bereavement (Sarper & Rodrigues, 2024), receiving social support particularly after this relationship loss may be crucial to prevent the development of severe grief experiences (e.g., PGD symptomatology) and improve quality of life (Park et al., 2023).

Given that public stigma predicts self-stigma later on (Vogel et al., 2013), our results highlight the need to raise awareness and develop tools to be used by mental health professionals to counteract public stigma. For example, our results could be used to inform the development of social media campaigns to increase awareness toward PGD and companion animal losses, by showing how these experiences tend to be stigmatized by society and their potential consequences for bereaved people. Complementing recent research on the disenfranchised nature of companion animal loss (Spain et al., 2019), our findings could also inform the development of modules to be included in courses and training sessions directed at professionals working with people who lost their companion animals (e.g., certified courses for psychologists and other mental health professionals).

Notes

1. PGD has also been termed as complicated grief, complicated grief disorder, and persistent complex bereavement disorder (Eisma, 2023; Lenferink et al., 2022). Criteria established on the Diagnostic and Statistical Manual of Mental Disorders (DSM-5-TR) for adults to be diagnosed with PGD include impairment in psychosocial functioning and the experience of severe symptoms (e.g., a sense of disbelief about the death) for more than 12 months (Prigerson et al., 2021). Similarly, criteria established by the International Classification of Diseases (ICD-11) for adults to be diagnosed with PGD include persistent longing for the deceased, intense emotional reactions (e.g., guilt), and functional impairment for at least 6 months (Shevlin et al., 2023).
2. We conducted two one-sided tests (TOST) using $-.05$ and $+.05$ Cohen's d as our interval of significance to determine the equivalence between both gender

conditions across public stigma indicators (Lakens et al., 2018). Results showed that means were significantly below the upper bound of the confidence interval, all $p \leq .003$, and significantly above the lower bound of the confidence interval, all $p \leq .001$, indicating that means were statistically equivalent for female and male targets.

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